BASELINE → TREATMENT → FOLLOW-UP

Checks across your durvalumab course

Relative planning stages, not booked appointments or equal time intervals. Follow the team’s actual blood-test and scan schedule; extra endocrine, lung or heart tests depend on symptoms and findings.

Check	Before durvalumab	First infusion	Later cycles	Between visits	After completion / a hold
Treatment sequence	Finish concurrent chemotherapy + radiotherapy	Start within the source window	Day 1 every 28 days	No durvalumab tablets at home	Up to 12 months / 13 cycles; review any restart
Blood count / kidney / liver	Baseline tests	Review available results	Regular tests under the team plan	Extra tests for symptoms or abnormalities	Follow abnormalities until resolved / stable
Glucose / thyroid / hormones	Baseline glucose and thyroid tests	Review symptoms and medicines	Regular glucose and thyroid tests	Targeted hormone tests when indicated	Ongoing replacement and follow-up if needed
Lung and other immune symptoms	Review CRT recovery and baseline breathing	Symptom check	Ask before each infusion	Report changes promptly	Delayed immune effects still need assessment
Hepatitis / tumour / scans	Hepatitis B/C screen; validated PD-L1	Confirm stage and no progression	Scans as the consultant plans	New symptoms may need earlier assessment	Agree follow-up and contact arrangements

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00655 v3a · official regimen · EMA Imfinzi product information · sections 4.2–4.6, 5.1

THE WHOLE TREATMENT JOURNEY

Durvalumab 1500 mg every 28 days: treatment and checks

Stage III NSCLC · consolidation after concurrent chemoradiotherapy

Durvalumab is infused on Day 1 of each 28-day cycle. The course lasts up to 12 months (13 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before durvalumab	First infusion	Later cycles	Between visits	After completion / a hold
Treatment sequence NCCP 00655	Finish concurrent chemotherapy + radiotherapy	Start within the source window	Day 1 every 28 days	No durvalumab tablets at home	Up to 12 months / 13 cycles; review any restart
Blood count / kidney / liver NCCP 00655	Baseline tests	Review available results	Regular tests under the team plan	Extra tests for symptoms or abnormalities	Follow abnormalities until resolved / stable
Glucose / thyroid / hormones NCCP 00655 · SMPC	Baseline glucose and thyroid tests	Review symptoms and medicines	Regular glucose and thyroid tests	Targeted hormone tests when indicated	Ongoing replacement and follow-up if needed
Lung and other immune symptoms NCCP 00655 · SMPC	Review CRT recovery and baseline breathing	Symptom check	Ask before each infusion	Report changes promptly	Delayed immune effects still need assessment
Hepatitis / tumour / scans NCCP 00655	Hepatitis B/C screen; validated PD-L1	Confirm stage and no progression	Scans under the agreed follow-up plan	New symptoms may need earlier assessment	Agree follow-up and contact arrangements

What each check is for

1 Finish CRT and confirm suitability

Before the first durvalumab dose

Chemotherapy and radiation are concurrent first; durvalumab follows after completion.

Ask / act: Review the actual treatment sequence, recovery and baseline tests.

Sources: NCCP 00655

2 Recognise lung and endocrine symptoms

Throughout the course, holds and follow-up

Immune inflammation and radiation effects can overlap; hormone problems can develop with subtle symptoms.

Ask / act: Report changes promptly and keep the alert card available.

Sources: NCCP 00655 · SMPC

3 Complete, pause or stop safely

At toxicity review or the maximum course

The course is capped at 12 months (13 cycles), with earlier stopping if needed.

Ask / act: Agree follow-up, hormone replacement if needed and how to obtain urgent advice after completion.

Sources: NCCP 00655 · SMPC

4 Agree follow-up imaging

After CRT, during treatment and after completion

Look for recurrence when further intervention would be appropriate.

Ask / act: Follow your team’s scan plan and report new symptoms without waiting for a routine scan.

Sources: IMAGING

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.26